Medication Instructions Before Colonoscopy

Kurt Vernon, MD, PA · The GI Guy

Procedure date: _____	**Arrival time:** _____
Procedure time: _____	**Stop all liquids at (office fills in):** _____
Blood thinner: medicine: _____	**Continue or hold:** _____
Last dose (date and time): _____	**Restart (date and time, or who will tell you):** _____
Plan confirmed by, and date: _____	

These are Dr. Vernon's practice instructions for colonoscopy with sedation. **Never stop a prescribed blood thinner on your own.** Ask the office for a written plan before your prep day, and call if anything here doesn't fit your situation.

Blood Thinners: Get a Plan Made for You

- **Antiplatelets:** clopidogrel (Plavix), prasugrel (Effient), ticagrelor (Brilinta) and similar medicines.
- **Anticoagulants:** warfarin (Coumadin, Jantoven), apixaban (Eliquis), rivaroxaban (Xarelto), dabigatran (Pradaxa), edoxaban (Savaysa), and injections such as enoxaparin (Lovenox).

Call our office and the doctor who prescribes your blood thinner. The plan depends on your procedure, your kidney function and why you take the medicine. Stopping it can cause a stroke, heart attack, stent clot or another serious clot. Tell us about a recent stent, stroke, blood clot or mechanical heart valve. Don't start "bridging" injections unless you're specifically told to.

Don't guess when to restart. Follow the written plan you're given after your procedure.

Baby Aspirin: Keep Taking It

Keep taking prescribed baby aspirin (81 mg a day) unless your GI doctor and the doctor who prescribed it give you a different plan. It is not part of the NSAID hold below. If you take a different aspirin dose or a combination product, call for instructions.

NSAIDs: Stop 7 Days Before

Don't take non-aspirin NSAIDs during the 7 days before your procedure, or on the day of it. Examples: ibuprofen (Advil, Motrin), naproxen (Aleve, Naprosyn), diclofenac (Voltaren), meloxicam (Mobic), indomethacin, ketorolac and celecoxib (Celebrex). Ask about creams, gels and combination products.

Some NSAIDs affect bleeding, and NSAIDs can also raise the risk of kidney injury when you're dehydrated from the prep. Acetaminophen (Tylenol) is not an NSAID; ask whether it's right for you, especially if you have liver disease.

GLP-1 and GIP Medicines: Stop at Least 7 Days Before

Stop these medicines at least 7 days before your procedure, and don't take them on the day of it. Examples: semaglutide (Ozempic, Wegovy, Rybelsus), tirzepatide (Mounjaro, Zepbound), dulaglutide (Trulicity), liraglutide (Victoza, Saxenda) and exenatide. Tell us about daily, weekly and compounded products.

Why: these medicines slow how fast your stomach empties. Food or liquid left in the stomach can come back up and get into the lungs during sedation. Stopping for 7 days doesn't guarantee an empty stomach, so tell us about nausea, vomiting, bloating, feeling full quickly, severe constipation or a recent dose increase.

This is our practice's policy; national guidance lets many patients keep taking these medicines after an individual review. If you take one for diabetes, set up a blood sugar plan before you stop it. Call if you took a dose within the last 7 days, and the team will decide what to do.

SGLT2 Medicines: Stop at Least 4 Days Before

Stop these medicines at least 4 days before your procedure, and don't take them on the day of it. Examples: empagliflozin (Jardiance), dapagliflozin (Farxiga), canagliflozin (Invokana), ertugliflozin (Steglatro), and combination pills that contain them. This applies even if you take them for your heart or kidneys.

Why: fasting and dehydration can trigger ketoacidosis, sometimes even when your blood sugar isn't very high. Get urgent care for vomiting, belly pain, fast or deep breathing, unusual weakness or confusion, and tell them you take an SGLT2 medicine. Restart only when your doctor says it's safe and you're eating and drinking normally.

Phentermine: Stop at Least 7 Days Before

Stop phentermine (Adipex-P, Lomaira) at least 7 days before your procedure, and don't take it on the day of it. It can affect your blood pressure, heart rate and how you respond to anesthesia. If you take phentermine with topiramate (Qsymia), call the doctor who prescribes it early for a safe plan; don't stop that combination suddenly on your own.

Insulin and Other Diabetes Medicines

Ask for a written plan for insulin, metformin, sulfonylureas and your other diabetes medicines. Check your blood sugar as directed on the clear-liquid day. **Never stop all insulin if you have type 1 diabetes.** If your blood sugar is low, treat it the way your diabetes plan says and tell the procedure team, even though you're fasting. Call if you missed a required hold.

Counting the Days

- **7-day hold:** your last dose must be at least 168 hours (7 full days) before your procedure.
- **4-day hold:** at least 96 hours (4 full days) before.

Ask the office to write down your exact last-dose dates. Restart GLP-1 medicines and phentermine only as directed; some medicines need a dose change after a break.

Bring this sheet and your medication list to your procedure. Questions: Fuquay-Varina (919) 577-0085 or Dunn (910) 891-5808.